

COURT FILE NO.: 00-BN-2565SR

DATE: April 28, 2003

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

MARILYN MANNING and DAVID
MANNING

Plaintiffs

- and -

3980 INVESTMENTS LTD.

Defendant

)
)
) J. Sebastian Winny, for the Plaintiff
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) D. Bierstone, for the Defendant
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) **HEARD:** April 1-4, 7-9, 2003
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REASONS FOR JUDGMENT

BELLEGGHEM J.

The Action

[1] The plaintiff sues for damages sustained when she slipped and fell on January 19, 1996 on premises occupied by the defendant.

Issues

[2]

- 1) Defendant's liability;
- 2) Plaintiff's contributory negligence;
- 3) Were the plaintiff's long-term shoulder difficulties "caused" in the fall, or "by plaintiff's employment";
- 4) General damages;
- 5) Assessment, if any, of the plaintiff's employment losses.

The Evidence – The "Fall"

[3] The plaintiff is a 59 year old married woman. She retired from a banking position after 22 years in 1994. She took some time off and decided to return to work doing factory labour. At the time of her fall, she was registered with Manpower doing odd jobs.

[4] She testified that on January 19, 1996 between 3:00 and 3:20 p.m., she had parked in the visitors' parking at her son's apartment at 2900 Battleford Road, Mississauga. She was carrying a container of spaghetti in her hands while her purse was slung over her left shoulder. She described it as a beautiful sunny day, mild in the morning but colder in the afternoon. She was wearing a ski jacket, jeans and Nike runners. Snow was piled up on the perimeter of the

parking lot which had not been salted or sanded. She exited her car in the visitor parking lot and was proceeding along a 15 foot patio stone sidewalk to the east door of the building.

[5] The walkway was one patio stone width (18 to 24 inches). Otherwise it was a flat open area. There were no stairs or handrails along the walk.

[6] Visibility was excellent. The walkway was all in shadow when she went into the building. She was able to observe the condition of the walkway from the parking lot to the east door. She did not see any ice or snow on the patio stones. Some of the stones, however, looked as though they had moisture on them as they were darker gray in spots. The path was straight. She could not tell the distance between the stones because they were too uneven. Some of them were separated by one to one and-a-half inches and others were closer together. Some of the stones were up and some were down in height from the others. She described the greatest height difference between the stones at about one inch, the depth of the stones themselves. Most of the stones were uneven according to her testimony. There was no salt or sand on the stones.

[7] She said that she stepped up off the curb, her right leg slipped and she went down on her right knee. The place where she slipped, she said, was the second or third stone in from the curb. She thought she was in the middle of the

third stone when she fell. She said that as she went to take a step, her foot slid out from under her.

[8] At the time she fell, she said she was looking straight ahead to the door area.

[9] In cross-examination, she admitted that she had used the walkway on which she slipped quite frequently. She said she noticed the stones were out of line. She looked down as she stepped onto the curb. She had seen details of the stones prior and also the day of the incident.

[10] She described the patch that she slipped on as being dark gray and looking like moisture. It was not until she fell and she felt it with her hand that she realized it was soft ice, the type that forms in an ice cube tray before it becomes glassy.

[11] She agreed that she had made notes a couple of weeks after the incident. She also agreed that she did not mention initially anything about the unevenness of the stones. In her notes, she only refers to a moist patch and she agrees that she did not, in fact, see any ice until she fell.

[12] The plaintiff's son, Brian Schram, testified that he had lived at the 16 floor apartment building for four years. It was moderately maintained and housed

what he described as “lower income” residents. The east door was used by residents who parked in the east lot.

[13] The pathway to the door was about 15 feet long, made up of two parallel lines of two foot square pavers. They had been uneven for some time causing puddling and they were eventually smoothed out and leveled the following spring. The fence was put in later to keep people off the grass and was not there when she fell.

[14] When she fell, she said she hit her knee, her side, her hip and her elbow on the ground or the cement. She landed on her hip and rolled onto her right elbow and arm. She said that her whole arm contacted the cement walkway.

[15] She lay there a while and could see that her jeans were torn and that her knee was bleeding. She thought she had the breath knocked out of her.

[16] She managed to get inside the east door and get to the superintendent's office. At the time, most of the pain was in her knee. She went to the office to inform the superintendent of what had occurred and to ask why the walkway had not been sanded. At the office, she met a lady who appears to have been the temporary superintendent's wife who then called the temporary superintendent. She was unable to converse with him because he did not speak English.

[17] The plaintiff testified that she had resided at the apartment building herself for a few months shortly before the incident and was familiar with a lady who did some of the cleaning. She explained to this lady what had happened and complained to the people in the superintendent's office that someone should immediately look after having the walkway salted or sanded. She saw bags of salt against the wall by the door in the office.

[18] After lodging her complaint, she got to her son's apartment. Her son, with the assistance of a lady friend, gave her some first aid. He said that his mother limped into his apartment the day of the incident with blood on her knee saying that she had slipped on some ice by the side of the building on the sidewalk. The east door was the closest entrance for her to come in from the parking lot. He complained to the building staff later that evening that the path had still not been salted (about 5:00 p.m.). He saw bags of salt on the floor of the maintenance office, including one that was opened and noticed that the front of the building had been salted but not the east side.

[19] Two and-a-half to three hours after her fall, her son escorted her to her car. His mother showed him the ice patch where she had fallen. He described it as a patch of ice two feet in diameter, eight feet from the curb of the parking lot, roughly in the middle of the sidewalk.

[20] She was able to drive home where she took some aspirins and went to bed about 7:00 p.m. (Her son contradicted her on a couple of minor points. He thought the motor vehicle accident she had been in was after her fall, and that she hadn't actually lived at the building but had stayed over on occasion.)

[21] She contacted people at the building the next day and was told that a Mr. Horvat would call her. She understood Mr. Horvat to be the property manager. She heard from him a short time later and told him how the fall had occurred.

The “Expert Evidence”

[22] The plaintiff called Sam Kodsi, a forensic engineer, who testified as an expert on human factors. He said that a half inch differential between pavers, particularly given fluctuating temperatures and freezing with the likely accumulation of ice, absent salt or sand or both would provide a path that “would not be reasonably safe”. He referred to new construction standards, in particular one that was promulgated the same month as the incident, that suggested beveling edges between one-quarter and one-half inch and ramping edge differences of more than a half inch. He said these standards were used as guide lines by human factors engineers, in assessing safe walking conditions.

[23] His opinion was challenged by Sean Jay, called by the defence. Mr. Jay also testified as an expert in human factors engineering. He was familiar with the standard referred to by Mr. Kodsi the plaintiff's expert but described it as "very stringent". He said it applied only to new construction or reconstruction, and not "maintenance" for which he was not aware of any standard. He also pointed out that it was not part of the Ontario Building Code. He said it was not reasonable to apply it the same month that it was promulgated.

[24] From an engineering standpoint, he testified that while the standard required beveling height differentials of one-quarter inch to one-half inch, and ramping beyond a half inch, that such height differences would not be unsafe.

[25] He placed the "unsafe" level, which he described as a "trip hazard", at one inch. He said that height differentials of one-quarter inch to one-half inch are common throughout the City of Toronto, and even in the pedestrian areas surrounding the courthouse in which the trial was taking place. His view was that any able bodied person would have no difficulty with the degree of unevenness testified to by the plaintiff and which was the subject of the plaintiff's expert opinion.

[26] He incorporated the weather data for Pearson Airport into his opinion. That particular day was somewhat unusual. The temperature had fluctuated

around the freezing mark prior to the day of the incident. On the morning of the incident, the temperature dropped dramatically below the freezing point. Winds picked up intermittently and there were snow showers through the day. There had been rain prior. It was reasonable to assume, therefore, that the unusual conditions would be apparent to both landlord and pedestrian. They could reasonably be expected to take steps to deal with the rapidly changing weather condition or, at the very least, take it into account, in the conduct of their affairs.

[27] He also suggested that “runners”, as described by the plaintiff as her footwear that day, may be less effective in slippery conditions than typical winter footwear.

[28] The experts’ opinions on the safety standards and the effect of uneven pathways was of limited value. I am satisfied that either the pathway was not as uneven as testified to by the plaintiff, (she never mentioned it very early on), or it was not so uneven as to have contributed significantly to the incident. As will be apparent in my subsequent analysis, I agree with defence counsel that this case is about slipping and falling on an icy surface. It is not about tripping, or slipping, on an uneven surface, even though it could reasonably be argued that unevenness can contribute not only to pooling, but to pedestrian stability on an icy surface.

[29] The fact that the plaintiff failed to allude to the pathway being uneven in her original notes satisfies me that this is either an afterthought or, as suggested by defence counsel, arose as a result of her review of the statement of Mr. Horvat. I recognize that this latter possibility was not put to her, but I am satisfied, even in the absence of Horvat's statement and defence counsel's resulting argument, that there was either no unevenness, or she would have alluded to it early, or even if there was, it was not sufficiently significant in her mind to have been a factor. As will be seen from my analysis, therefore, I treat this case as a relatively straight forward "slip and fall" on an icy sidewalk in the course of an unusual day, weatherwise.

[30] Before leaving this part of the evidence, I am also satisfied that the plaintiff's description of it as being a sunny day, is perfectly consistent with the meteorological data that it was windy with snow showers on and off through the day. I also find that the conditions at the airport were the same as at the scene.

Building Maintenance

[31] John Horvat testified that his company "Regional Group" had been retained by the defendant corporation to manage the building about three weeks before the incident. The building had, according to Mr. Horvat, been "run into the ground" by the previous landlord. From his evidence I infer that it was in some

degree of disrepair. His company, in turn, hired “Approach” maintenance to look after the building until a permanent superintendent could be obtained.

[32] Mr. Horvat was advised of Ms. Manning’s fall by the superintendent and referred the matter to the insurer. Horvat spoke with the plaintiff but was not sure where the fall took place. In his statement to the insurance company, he refers to a walkway on the south part of the building which had become uneven through disrepair and because of trucks backing over it. However, this is not the same walkway where the incident occurred. The incident occurred on the 15 foot walkway on the east side of the building leading from the visitors’ parking to the east fire exit. He could offer no evidence as to the condition of the walkway where the incident occurred.

[33] George Papadimitriou was what I find to be the “temporary building superintendent”, acting through the above-noted chain ultimately for the landlord/defendant. There were so many contradictions in Mr. Papadimitriou’s evidence that it was difficult to follow. He describes snow moving as his priority. He said that he had cleared the snow once at 7:30 in the morning and again around 10:00 a.m. He said that the plaintiff came in to see him about 10:30, just after he had finished clearing the snow for the second time. He described the weather as bad that day and that “it snowed a lot”.

[34] This is simply inconsistent with the testimony of any other witness and with the meteorological evidence. His evidence respecting the time of the event is also drastically inconsistent with the evidence of the plaintiff and her son respecting the time. He talks about “plowing” the snow, while his wife is throwing salt right behind him as he goes. He also testifies about the plaintiff taking him to the place where she fell. She, of course, denies that she showed him where she fell but rather that he simply left the office when she made her complaint and she never saw him again.

[35] He adds embellishment to this fictitious description when he says that the place where she showed him she fell had salt on it, but no snow or ice. He also has some difficulty in pinpointing the area as being either on the walkway near the curb to the visitor’s parking lot, or even onto the parking lot area itself. This is significant because he testified that he was not responsible for keeping snow off the parking lot itself or for salting it, although he said that when he salted the walkway some salt would fall onto the parking areas.

[36] From the foregoing, it is obvious that his evidence is of very little assistance except to the extent that it verifies that the weather was “bad”, though I find that it was not snowing all day. The weather was described in the meteorological data. There was a rapid freeze after intermittent freezing and

thawing, intermittent sunny with snow showers, resulting in trace amounts of snow, and gusts of wind.

[37] Mr. Papadimitriou's boss, Danny Siatskas, testified that his property management company was on the site for only a few weeks until a permanent superintendent was obtained. Mr. Siatskas was the one who gave Mr. Papadimitriou his instructions respecting the snow. He told him to clear the snow and throw salt when it snowed on all of the walkways. His company was not responsible for doing the parking lot. His evidence respecting the time of event was somewhat inconsistent and conflicting with the evidence of the plaintiff. Initially, he said that Mr. Papadimitriou phoned him before lunch and that he arrived about 1:30 or 2:00. Ultimately, he said that Mr. Papadimitriou told him the incident had happened five or ten minutes ago and that he was there within 40 or 50 minutes, about 2:30 or 3:30 in the afternoon. His evidence that it was bright at 2:30 or 3:30 in the afternoon is consistent with the plaintiff's evidence.

[38] He said that Mr. Papadimitriou showed him where she had fallen on the parking lot close to the sidewalk. The place that Mr. Papadimitriou showed him was not on the east sidewalk itself. He said the sidewalk appeared freshly salted when Mr. Papadimitriou was showing it to him. He also said that he made it clear to Mr. Papadimitriou that snow and ice were the number one priority, that all of the walking areas around the building were to be kept clear, including the east

sidewalk. He could not assist with respect to whether or not the east sidewalk was crooked or uneven. As I indicated earlier, and will do so later in my analysis, the issue of unevenness and crookedness is a “red herring”.

[39] The spot that Mr. Papadimitriou showed him as the spot where Ms. Manning fell, ie., on the parking lot, did not have snow and ice on it and appeared freshly salted. I am satisfied that the fall took place on the sidewalk in the vicinity indicated by the plaintiff. While Mr. Papadimitriou may well have shown Mr. Siatskas a spot on the parking lot freshly salted with no snow or ice, I am satisfied that the fall did not occur on the parking lot. I have already rejected the evidence of Mr. Papadimitriou as unreasonable, inconsistent, conflicting and unreliable.

The “Work Related” Injury

[40] In September of 1996, about eight months after her fall, the plaintiff managed to secure full time employment with Glaxo as a packager of pharmaceutical supplies. She worked on an assembly line putting supplies in boxes and boxes onto skids. She worked 40 hours a week, three different shifts from September 1996 until November of 2000. Generally the work was very light unless she worked in “topical” when she would have to do some heavy lifting. She also had to do some cleaning of the ceiling using long-handled mops.

[41] From October 2000 through November 2000, the assembly line work became somewhat hectic because she and a co-employee were required to work extra hard on the line while training new workers. During that month, she got increasing pain down her right arm and shoulder. She had not sustained any shoulder or arm injuries prior or subsequent to the January 19, 1996 fall and her neck, shoulder and arm had never fully recovered from the fall when she found herself in the work-related scenario in October and November of 2000.

[42] As the pain got worse during that month, she chalked it up to old age. She saw Dr. Poon. He gave her a prescription and referred her to Dr. Weisleder again. She was taking anti-inflammatories but she still could not move her arm. Her shoulder movements were restricted. She was in pain. She was sent for an MRI in January of 2001 to see what the difficulty was with her shoulder and arm. Ultimately, in February of 2001, Dr. Weisleder performed an operation on her shoulder to free up some tendons, although it was originally thought she had suffered a rotator cuff tear. After the operation, she had a “frozen shoulder”. She had a lot of severe long lasting pain following the operation. She was unable to use her right arm. She returned to physiotherapy which continued until December of 2001.

Long Term Problems

[43] At the end of physiotherapy, she said that she was fine except for the pain, and that she is limited with what she can do with her arm. She demonstrated difficulty in putting her arm behind her back. She is unable to put dishes in a cupboard. She has trouble making beds. She cannot vacuum. Although she has a little more range of her arm now, she is still not 100%. She last saw Dr. Weisleder in February of 2003. She was told to call him if her arm gets worse.

[44] She demonstrated a scarring on her right knee. Photographs of the scars are filed. She considers the scars to be “ugly”. There is a one and-a-half inch line on the knee cap, light in colour, and a one and-a-half inch square patch about three inches below the right knee cap.

[45] At the present time, she says her right shoulder is constantly sore. The pain fluctuates in intensity. Sometimes it is just a dull throbbing pain. Movement of the right shoulder is restricted. A muscle pulls in her upper and lower arm when her arm is raised too high. She cannot put her arm behind her back. She cannot sleep on her right side. She cannot raise her arm more than 45 degrees above her shoulder without tightness and pain.

Employment

[46] In 1993 and 1994, in her banking job, she made between \$26,000 and \$29,000 per year. Her income was reduced in 1995, as she took a retirement package.

[47] In 1996, her employment income was \$3,256. In 1997, when she was full time with Glaxo, her employment income was \$11,966. She was comfortable with the hours and pay.

[48] She made roughly the same from 1998 through 2000. The 2000 income was supplemented by employment insurance. The 2001 income was primarily employment insurance, roughly \$12,000.

[49] She was able to return to work in February of 2002 doing some light office work with Glaxo. They did not have a lot of work for her and her doctor would not let her work on the assembly line. Eventually, she was let go from Glaxo because they did not have enough work for her.

[50] After leaving Glaxo in July of 2002, she has been looking for work. Presently, she is hoping to get employment which will provide fewer hours, but roughly the same pay, at a group called "Seniors helping Seniors".

Liability – Analysis

[51] I accept the plaintiff's evidence that the incident occurred around 3:00 o'clock in the afternoon. At the actual time that she fell, it was likely sunny, although there were intermittent snow showers throughout the day. It was also windy. The temperature had dropped quite dramatically that morning, following temperatures which fluctuated around freezing. The conditions were unusual for January. The conditions would be equally obvious to both the superintendent and the plaintiff.

[52] The plaintiff was likely wearing rubber soled Nike runners. She was carrying a 5 pound container of spaghetti, likely in two hands. She also had her purse strung by a strap over her left shoulder. She was familiar with the building, and had used the walkway many times before. As she testified, she was looking down when she stepped onto the walkway from the parking area. She saw a dark gray patch. She took this to be moisture. She did not describe seeing any salt which could have accounted for there being moisture rather than ice or slush. The consistency of the patch, she later testified, was similar to the beginning of freezing on the top of an ice cube tray. I am satisfied that there was no salt applied at the location described by her at the time she fell.

[53] The conditions, weatherwise, were particularly treacherous. However, this would be known to her. She ought therefore to have taken greater care for her own safety. She erred in misjudging the patch of dark gray as being

moisture, which presumably she thought wouldn't be slippery, rather than ice or slush.

[54] The walkway was likely commonly used by persons who parked outside, either residents or visitors, if they had a key. The key would fit all of the doors. It would more likely, therefore, be used, or expected to be used, by residents. Residents would be familiar with the walkway. Ms. Manning was familiar with the walkway.

[55] Her original notes do not indicate anything respecting the walkway being uneven, nor any unevenness in the walkway contributing to her fall. It is simply a case of her having misjudged what the substance was that she was stepping on, when it was there to be seen. In the circumstances, it ought to have been known by her that it would be slippery. As defence counsel points out, one wonders why, if she thought it was moisture, she would not have stepped on the lighter gray area in any event.

[56] The temporary superintendent and the persons who hired him were all acting under the ultimate control of the landlord, and on the landlord's behalf. Their duty was to "take such care as in all of the circumstances of the case is reasonable to see that persons entering on the premises ... are reasonably safe while on the premises".

[57] Section 6(1) of the *Act* does not apply to relieve the landlord of his liability in the present case.

[58] The property had newly been taken over by the landlord. Evidence called on behalf of the defence satisfies me that the property had been neglected. Mr. Papadimitriou was only there on a temporary basis. While he got his instructions from Mr. Siatskas, there was no written protocol or record of salting or sanding. There was no written instruction.

[59] Mr. Papadimitriou's evidence is very unsatisfactory for the reasons touched on by me when I reviewed it. I simply reject his evidence that he had removed the snow, and had his wife throw salt. Meteorological records show that there was simply not enough snow to be moved by a snow blower or "plow" as described by him. He is misleading or mistaken about this. His evidence is totally unreliable. The meteorological evidence, which is more objective, is consistent with the evidence of the plaintiff and her son as to the condition. I am satisfied that the east walk had not been salted.

[60] I am not satisfied that any crookedness or unevenness caused or contributed to the fall. The fall was caused by a combination of slipperiness, which was unreasonably permitted to continue in spite of the obvious nature of the treacherous conditions, and by the failure of the plaintiff to take care for her

own safety when the conditions were obvious to her, and observed by her, although misjudged by her.

[61] Under s. 6(1) of the *Act*, the defendant “acted reasonably in entrusting the work to the independent contractor”, but failed to take “such steps ... as the occupier reasonably ought in order to be satisfied that the contractor was competent, and that the work had been properly done ...”

[62] There was simply an inadequate system of instruction, and supervision, of the temporary superintendent, by those over him, leading in due course to the landlord. There was no system in place to ensure salting or sanding. There was no system in place to ensure when and by whom it was being carried out. There was no system in place for follow up, particularly when conditions became treacherous.

[63] Mr. Papadimitriou may well have removed the snow on prior occasions. He may well have salted on prior occasions. I am satisfied on the basis of the plaintiff’s evidence and that of her son that on this particular occasion he did not see any snow that needed to be removed. He did not inspect the walkways, particularly the east walkway, because there was no protocol requiring him, put in place by his seniors, to do so. There was absolutely no protocol in place, contrary to what the defence witnesses say, to determine whether the temporary

superintendent was tending to the walkways as conditions deteriorated throughout the early morning, and later on into the day, when the incident occurred.

[64] As long ago as 1957, the Ontario Court of Appeal in *MacIntosh v. Hillman and Turnbull Elevator Company Ltd.*, [1957] O.R. 284 stated the extent to which the landlord can avoid liability by contracting his occupiers liability out to a third party.

P. 18, Quicklaw Report:

... he warrants, however, that due care and skill to make the premises reasonably safe for the invitee have been exercised, whether by himself, his servants, or agents or by independent contractors whom he employs to perform his duty. He does not fulfill the warranty merely by leaving the work to contractors, however reputable or generally competent. His warranty is broken if they fail to exercise the proper care and skill. ...

[65] The principle was recently reiterated in the Supreme Court of Canada in *Lewis v British Columbia*, 1997, 3 S.C.R. 1145. At paragraph 19 the court says this:

In some circumstances, the duty to take reasonable care may well be discharged by hiring and, if required, supervising a competent contractor to perform the particular work. The standard of reasonable care is met by exercising reasonable care in the selection and, in some situations, the supervision of an independent contractor qualified to undertake the work. If this is done, then the principal will usually not be held liable for injury caused by the negligence of the

independent contractor. This long standing principle of the common law is today reflected in many of the occupiers' liability statutes enacted by the provinces. These Acts generally protect the occupier from negligence of an independent contractor qualified to perform the work. [Underlining mine]

[66] The *Ontario Occupiers Liability Act*, s. 6(1) incorporates this principle as a defence. The defence, however, is only available if the evidence supports it. In the present case, I have already found that the landlord and agents acting on his behalf failed to exercise reasonable care in the selection and supervision of the temporary superintendent. In particular, the supervision in the present case, I find, was either absent or minimal at best. At the very least, it was unreasonable. Accordingly, the landlord defendant is liable for the failure of the superintendent to take the care required under s. 3(1) of the *Act*, by failing to have an up-to-date inspection system, by failing to inspect the east walk, and by failing to salt or sand or both, as required. It was clearly required on this particularly unusually treacherous day. However, the matter of liability does not end there.

[67] Then J., June 20, 1991, in *Bogoroch v. Toronto (City)*, [1991] O.J. 1032 (Gen. Div.) alluded to the following circumstance which is relevant in the present case:

...the presence of snow and ice on sidewalks in Canada is a common occurrence in the month of February...the occupier is not an insurer required at all times to keep his premises absolutely free of snow and ice but rather the

standard of care required is that the occupier take reasonable steps to keep his premises safe in the circumstances. ...

[68] Justice Then then went on to find that the plaintiff in that case was 40 per cent responsible for his own “failure to keep a proper lookout”. In that case,

The plaintiff did not notice the presence of ice on the roadway when he exited his car nor did he focus his attention onto the sidewalk but rather his attention was focused on the door of the store. ...

[69] In the present case, the evidence of the plaintiff is that she indeed focused her attention on the sidewalk. Her negligence does not consist in failing to keep a proper lookout in the same manner as the plaintiff in the *Bogoroch* case. Her negligence consists in an error in judgment. She erred unreasonably when she assumed that what she saw as some “moisture” was safe to step on without risk of slipping when it ought to have been readily apparent to her that “moisture” in such circumstances was more likely than not to be slippery, icy slush, given the weather conditions which had existed up to that point in time.

[70] She was in a place where it was likely shaded, being on the east side of a tall apartment building. The temperature had dropped rapidly. It had rained the previous night. There were intermittent snow showers. There was no salt on the walkway which was visible. It was more likely than not that what she

perceived as a dark gray patch was slippery simply because it was wet and, therefore, ice rather than dry and, therefore, less slippery.

[71] She also erred in stepping onto it after misjudging what it was, when there was nothing to indicate that she could not have walked in such a way as to avoid it. This is not a case where she suggests that the entire walkway was slippery. It appears to be only the dark gray patch that she stepped on that was slippery. She misjudged what it was, although she had all of the clues that she needed to make that reasonable assessment. She should not have stepped on it because she ought to have known that it was likely to be slippery.

[72] In the present case, therefore, I am satisfied that there was contributory negligence on the part of the plaintiff.

[73] While the plaintiff was contributorily negligent, I am satisfied that the greater portion of liability rests with the defendant, through the superintendent, who had the initial obligation to have a system in place to prevent the type of situation confronting the plaintiff, and lots of time to carry it out. It was 3:00 o'clock in the afternoon by the time she fell. The treacherous conditions had been ongoing. The walk had not been salted or sanded. Salt was available. It was not used.

[74] In these circumstances, therefore, I apportion liability; 60 % against the defendant. I find that the plaintiff is 40 % responsible for her own injuries.

Damages - Evidence

[75] Dr. Poon, the plaintiff's family doctor, saw her first on January 24, 1996 with respect to the injuries. She complained of pain to her right knee, her back, her right thigh, and her right "neck/sh", meaning her right shoulder. He prescribed anti-inflammatories, gave her a tetanus shot and arranged for x-rays and physiotherapy.

[76] On January 30, her main complaints were her right neck and right shoulder. There was no change to her right knee. She had started physiotherapy. He diagnosed continuing soft tissue injury.

[77] February 7, 1996, her main complaint was right neck pain going down her upper back. The knee was still a problem.

[78] February 21, 1996, she had the same complaints, though less concern with her knee, which was responding to physiotherapy. The abrasions took six to eight weeks to heal.

[79] By March 20, 1996, her right knee was improving, but her neck and upper back showed no improvement.

[80] April 17, 1996, she still had pain to her neck and upper back and was not improving except for her knee which appeared to have healed. X-rays were negative.

[81] Dr. Poon referred her to Dr. Weisleder. She said that Dr. Poon did not tell her that Dr. Weisleder was a specialist. She did not go to him because she did not see any point in it because she felt nothing more could be done.

[82] She agrees that she did not return to see Dr. Poon until May 5, 2000, more than four years after her last visit with him. At that time, she was complaining of persistent pain on the right side of her neck to her upper back, and some pain in her right knee.

[83] He diagnosed mild to moderate patellofemoral pain from damaged cartilage behind the knee cap. By this time, her counsel had written Dr. Poon saying that the file was going to be pursued energetically, and indirectly requesting a report connecting the fall to her present complaints.

[84] At that time, Dr. Poon did not make the connection between her fall and her neck pain in May of 2000. He testified that since then he has reviewed her history and realized now that there was damage to the rotator cuff which he described as a "tear". However, he ultimately retracted this when it was pointed

out that the surgeon who ultimately operated on her, Dr. Weisleder, did not find the “tear” that had been suggested by the MRI.

[85] (She agreed that she had been in a motor vehicle accident in 1994 and reported at that time sustaining an injury to her neck, shoulder and hip on the left side. It did not leave her with any permanent difficulties.)

[86] Dr. Poon saw her from May through November of 2000 with respect to her neck pain and continuing knee complaints. November 13, 2000 she told him that she was unable to work because of her inability to raise her arm. He, therefore, submitted a claim to the WSIB on her behalf.

[87] In her report of injury to the WSIB, she describes the “parts injured” as her right shoulder, elbow and wrist. She describes how the injury happened related to her packing job and having to maintain a fast pace. The report also reads “prior similar injury conditions”...”no”.

[88] In cross-examination she agreed that she told Dr. Weisleder in February 2001 that she injured her shoulder at work and that she had no prior problems with it. She also agreed that she saw Dr. McKee at St. Michael’s Hospital shoulder clinic August 16, 2001. His report indicates that she injured her shoulder October/November of 2000, and that she reported not recalling any precipitating bout, but that it came on gradually.

[89] She said it never crossed her mind about falling down. Everything she told the doctors was true. She said he asked her specifically when she noticed the pain and she was simply answering his questions.

[90] A WSIB assessment entitled “patient’s history of injury” gives an October 16, 2000 report of “pain in the shoulder and increased workload”. She agrees she told WSIB that she injured her shoulder at work. She agrees that she did not tell them that she had any prior shoulder problems.

[91] A January 18, 2001 WSIB memo suggests that she told them that she only injured her knee in the fall. WSIB accepted her shoulder claim. She was paid about \$2,000.00 in the year 2000, together with an 11% disability pension, and benefits for all of 2001.

[92] She explained away the failure to relate the fall to her shoulder when dealing with WSIB on the basis that she thought an injury to her shoulder meant “something dropping on me or breaking my collarbone”.

[93] While there is passing reference to an injury to her shoulder in the WSIB file, most of the commentary in the file supports the defendant’s argument that neither she nor her doctors were connecting her fall in 1996 to her work related repetitive strain injury for which her claim was made to WSIB in November of 2000.

[94] Dr. Poon suggested in his testimony that he could connect the fall with her repetitive strain injury complaints on the basis that the original injury could cause stretching and swelling of the tendon which would have “materially contributed” to the rubbing and irritation which ultimately resulted in the repetitive stress injury for which the WSIB claim was made.

[95] Dr. Ezra Silverstein, a prominent orthopedic surgeon called as an expert by the defendant, takes strong issue with this suggestion. His position is that the repetitive strain injury suffered by her in November 2000 was “truly new”. All of her medical examinations up until October of 2000 showed that she did not have limitation of range of motion of her shoulder nor did she have pain on motion. Had there been some sort of injury to the rotator cuff as suggested by Dr. Poon, it would have become inflamed or swollen and manifested itself at the time.

[96] The RSI, on the other hand, does not have a definitive onset date or precipitating event. It comes on in the manner that she described as a result of the repetitive nature of the work that she was doing. According to Dr. Silverstein, the symptomology described by her following the accident was inconsistent with the symptomology described following the repetitive strain in November of 2000. The pain was almost invariably commented on by Dr. Poon as being in the region of the trapezius. Had there been an actual injury to the rotator cuff, her symptoms would have been described by her and noted by Dr. Poon differently.

Dr. Silverstein describes the manner of Dr. Poon's reporting of her symptoms as the "classic cervical strain symptoms" with referred pain through the trapezius muscle from the neck to the upper shoulder. At no time was there a complaint of anterior shoulder pain or restriction of shoulder motion, the classic symptomology of a rotator cuff injury.

[97] In fact, according to Dr. Silverstein, his review of the medical records shows a completely different constellation of complaints between the injury in January 1996 and the repetitive strain incident in November of 2000.

[98] Dr. Poon agreed that she could get a repetitive strain injury from the type of work that she was doing. In fact, he supported her application to WSIB on this basis. On February 15, 2001, the plaintiff's counsel wrote to Dr. Poon requesting in somewhat forceful terms that he consider whether he could connect her slip and fall incident in January of 1996 to her condition in late 2000 and early 2001, which led to the operation on the shoulder by Dr. Weisleder.

[99] On February 21, 2001, Dr. Weisleder performed an open acromioplasty and partial resection of her distal clavicle with exploration of her rotator cuff. While he observed significant rotator cuff tendonitis, he did not identify a rotator cuff tear. The archway over the rotator cuff was enlarged and some bony material removed to provide greater shoulder mobility. While she recovered as

well as might be anticipated from this type of surgery, she still has restriction of motion and is unable to work on the assembly line. She is, however, capable of doing light work and earning an income equivalent to what she was making at the time of leaving Glaxo.

[100] While Dr. Poon concedes that the bulk of the WSIB file does not mention any association with the slip and fall incident in January 1996, he said that his focus was on her work-related condition. He agrees that on November 2, 2001, when he got around to sending a report to her lawyer, that he connected the 1996 injury to her present complaints, although he had not done so until that point.

[101] It is of some significance that Dr. Weisleder in his report of December 7, 2000, refers to her neck injury in the slip and fall and that she has “gone on to develop some residual neck pain and stiffness”. He adds, “she denies previous shoulder problems”.

[102] She also saw Dr. McKee at St. Michael’s hospital. In his report of August 16, 2001, some six months after the surgery, he notes that she “does not recall any specific traumatic precipitating event but rather a gradual increase and discomfort with repetitive activity”. He also notes her “significant neck

discomfort” and that “continued attention should also be paid to her cervical spine”.

[103] In Dr. Poon’s draft report of December 22, 1996, Dr. Silverstein notes that there is no report of limited shoulder motion nor anteriolateral pain. If there had been, then there may have been a suggestion of rotator cuff involvement. However, all of the pain is referable to the cervical strain. Had there been anteriolateral pain or limited shoulder motion this would have been a “significant finding”, and Dr. Poon would have noted it.

[104] Dr. Silverstein points out that the plaintiff’s story is essentially the same until November 2000. She simply did not complain of pain in her shoulder that was consistent with an injury in the slip and fall in January 1996 to her rotator cuff, which was the reason for her operation and continued limitations.

[105] The WSIB file, on the other hand, together with Dr. Poon’s notes following her referral to WSIB support a finding of a repetitive stress injury from her work. In addition, her description of the onset and the nature of the work that she was doing support the claim for a pension which she made to WSIB.

[106] Dr. Silverstein testified that movement of her neck would aggravate neck pain which is what occurred. He also said that if the pain was emanating from her shoulder, that it would be aggravated by moving the shoulder and that

there would be, in addition, a limited range of shoulder movement and pain on moving the shoulder. According to Dr. Silverstein, there is no indication of complaint or abnormalities related to the right shoulder until November of 2000.

[107] With respect to her knee, he found on his examination pain on compression of the patella, but there was no chondromalacia. There is, therefore, some residual patello-femoral pain but no significant wearing of the intra-articular cartilage. Her knee is functioning normally.

[108] He described the abrasions or scarring on her knee as hardly visible. X-ray of the right knee showed that it was entirely normal and identical to the left.

[109] He concluded that in her slip and fall of January 19, 1996, that she fell onto her knee and sustained patello-femoral impact which has left some residual minor patello-femoral pain. In addition, she sustained a mild cervical strain. She did not, however, sustain any injury to her right shoulder.

[110] Dr. Silverstein did not consider the cervical strain to be "significant". I disagree. Dr. McKee refers to "significant" neck discomfort. Her cervical complaints are ongoing, in 2001. Five years after her fall, I expect she has permanently damaged her cervical spine.

[111] When Dr. Silverstein examined her, she had right shoulder problems related to her repetitive strain injury in November of 2000. She had some “residual” complaints with respect to her cervical spine, according to him, and some mild patello-femoral pain as well as some minimal scarring. I accept his finding that she was not impaired in her day to day activities and she was capable of returning to work.

Damages – Analysis

[112] The plaintiff sustained a significant cervical strain, abrasions to her knee, and some mild scarring. However, her most significant claim relates to her right shoulder. I have reviewed the medical evidence. I agree with counsel for the plaintiff that the issue with respect to her right shoulder is whether the condition for which she was operated on was “caused” in the slip and fall.

[113] The Supreme Court of Canada sets out the “causation test” in the decision of *Athey v. Leonati*, 140 D.L.R. (4th) 235. At paragraph 15 the court said:

... that causation is established where the defendant's negligence “materially contributed” to the occurrence of the injury: ...

...

It is not now necessary, nor has it ever been, for the plaintiff to establish that the defendant's negligence was the sole cause of the injury.

In addition,

...the plaintiff must prove that the defendant's tortious conduct caused or contributed to the plaintiff's injury. The causation test is not to be applied too rigidly. Causation need not be determined by scientific precision ... it is "essentially a practical question of fact which can best be answered by ordinary common sense". Although the burden of proof remains with the plaintiff, in some circumstances an inference of causation may be drawn from the evidence without positive scientific proof.

Finally,

Causation is established where the plaintiff proves to the civil standard on a balance of probabilities that the defendant caused or contributed to the injury.

[114] Dr. Poon says the fall "materially contributed" to the shoulder condition for which Mrs. Manning was operated on. Dr. Silverstein says that the fall did not "materially contribute" to the condition.

[115] At the outset I do not approach the resolution of this issue by treating it as a contest between experts. The condition for which she was operated on was "rotator cuff impingement". It is described by Dr. Silverstein as a self-perpetuating process. Dr. Poon on the one hand was the immediate treating

physician and therefore the one initially in the best position to diagnose. However, an M.R.I. was done. Dr. Weisleder looked specifically to see if there was a rotator cuff tear as suggested by the M.R.I. He did not find one. He did find, however, rotator cuff tendonitis. Dr. Silverstein says that this is as a result of the repetitive strain injury. Dr. Poon, Dr. Weisleder and Dr. McKee all, implicitly or explicitly, consider the condition for which she was operated on to be referable to repetitive strain. The real issue then becomes whether or not there was some kind of an underlying condition which predisposed the shoulder to the condition, and whether this underlying condition was “caused” or “materially contributed” to by the slip and fall.

[116] A careful review of Dr. Poon’s notes, and Mrs. Manning’s complaints in light of the opinion of Dr. Silverstein satisfy me that the slip and fall resulted in a cervical strain injury. It did not result in any injury, or predisposing condition, to the rotator cuff which was the subject of the condition for which she was operated on. I am satisfied that the slip and fall did not “materially contribute” to the rotator cuff impingement which resulted in her operation and her long term shoulder restriction.

[117] I am satisfied that she sustained a moderately significant cervical strain, permanent mild patello-femoral pain, without chondromalacia, and some scars,

which, while objectively relatively minimal, are likely as bothersome to the plaintiff as she testifies.

[118] The plaintiff's position is that if the shoulder claim is not compensable, that the plaintiff's general damages are in the range of \$25,000 to \$50,000. Defence counsel suggests that general damages should be assessed in the range of \$5,000.

[119] I am satisfied that the cervical strain has caused on-going and permanent difficulties for the plaintiff. I am satisfied that the same can be said for both the scars to her knee and the pain under the kneecap. I assess her general damages at \$20,000.

[120] With respect to the special damage claim, I fix the OHIP subrogated claim at \$196.45, being the cost for services rendered between January 24, 1996 and May 1, 1996 inclusive.

[121] I agree with plaintiff's counsel that she would likely have worked part-time for half of the three months following her slip and fall. Her rate of pay at the time was \$8.50 per hour. Six weeks at 24 hour per week at \$8.50 per hour works out to \$1,224 which is the sum in which I fix her past lost income.

[122] As I have found that she has not lost any time from work subsequent to returning in 1996 as a result of the injuries I found her to have sustained in the 1996 accident, I apportion no further income loss.

Adverse Inference – Spoliation

[123] Plaintiff's counsel pointed out that the adjuster for the defendant's insurer lost photographs which were taken of the sidewalk in question. He asks that I make an adverse inference against the defence arising from the loss of the photos based on the theory of spoliation. I decline to do so. Photographs could just as easily have been taken by the plaintiff. She made notes within two weeks of the incident. She must therefore have contemplated taking some sort of action. It appears as though she had contact with counsel early on. I see no reason why her interest could not have been protected by having photographs taken herself. There is nothing in the conduct of the defendant that suggests that the loss of the photographs was intentional. Given the effluxion of time, it is not surprising that the photographs became misplaced. I decline to exercise my discretion in favour of the argument advanced to draw the adverse inference requested.

[124] The plaintiff argued that remedial measures taken by the plaintiff by way of leveling the sidewalk should play a part in the assessment of liability. In

this particular case I am satisfied on the evidence that the so called “remedial measures” were really part of an on-going process of keeping the walk way in safe condition, rather than it being evidence that the defendants are thereby conceding that it was unsafe. That is a routine maintenance issue, demonstrating that the defendant, subsequent to taking over the property, and at the earliest moment i.e. the spring of 1996, took steps to maintain the walkway in good order. I have already found that it was not in disrepair, save for the lack of salting and sanding, at the time of the plaintiff’s slip and fall.

Conclusion

[125] Judgment will issue in accordance with the above findings. The parties may make an appointment through the trial office to speak to the issue of costs. I would anticipate that the costs hearing should not exceed one half hour maximum.

BELLEGGHEM J.

Released: April 28, 2003

COURT FILE NO.: 00-BN-2565SR
DATE: April 28, 2003

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

MARILYN MANNING and DAVID
MANNING

Plaintiffs

- and -

3980 INVESTMENTS LTD.

Defendant

REASONS FOR JUDGMENT

BELLEGHEM J.

Released: April 28, 2003